

1. Administrative & Study Identification

Study Title: Study of Durvalumab Given With Chemoradiation Therapy in Patients With Unresectable Non-small Cell Lung Cancer
Official/Full Study Title: A Phase III, Randomized, Double-Blind, Placebo-Controlled, Multicenter Study of Durvalumab Given Concurrently with Platinum-Based Chemoradiation Therapy in Patients with Unresectable, Stage III Non-Small Cell Lung Cancer
Short Title/Acronym: PACIFIC-2 **Protocol Number:** D933KC00001 **NCT Number:** NCT03519971 **Sponsor/Company Name:** AstraZeneca
Investigational Product: Durvalumab (Imfinzi) **Phase of Development:** Phase III **Medical Monitor:** AstraZeneca Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To assess the efficacy of simultaneous immunotherapy (durvalumab) plus concurrent chemoradiotherapy (cCRT) followed by consolidation immunotherapy in terms of Progression-Free Survival (PFS) in patients with unresectable stage 3 non-small cell lung cancer (NSCLC).

Secondary Objectives: To evaluate Overall Survival (OS), Objective Response Rate (ORR), Duration of Response (DoR), disease control rate, time to death or distant metastases, pharmacokinetics, and safety/tolerability (including incidence of pneumonitis).

2.2 Background & Rationale While consolidation durvalumab following platinum-based chemoradiotherapy (the PACIFIC regimen) is the established standard of care for patients with unresectable, stage III NSCLC, it is unknown whether starting immunotherapy concurrently with definitive cCRT provides earlier synergistic anti-tumor immune responses. This study evaluates whether introducing durvalumab simultaneously with cCRT improves clinical outcomes (PFS and OS) compared to cCRT alone.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Placebo-controlled **Blinding:** Double-blind **Randomization:** 2:1 (Durvalumab + cCRT : Placebo + cCRT)

3.2 Planned Enrollment & Duration Target **SN\$:** 328 participants (Targeted Enrollment) **Trial Status:** Active, not recruiting **Number of Sites:** Multicenter (Global) **Estimated Study Start:** 29-Mar-2018 **Estimated Primary Completion:** 07-Sep-2023

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age ≥ 18 years. 2. Histologically or cytologically confirmed diagnosis of locally advanced, unresectable Stage III Non-Small Cell Lung Cancer (NSCLC). 3. World Health Organization (WHO) or Eastern Cooperative Oncology Group (ECOG) performance status of 0 or 1. 4. At least 1 measurable lesion not previously managed with radiation (per RECIST v1.1). 5. Life expectancy of ≥ 12 weeks at the time of randomization.

4.2 Exclusion Criteria 1. Prior therapy with an anti-PD-1 or anti-PD-L1 antibody. 2. Mixed small and non-small-cell histologic features. 3. Unresolved toxicity $\geq$ Grade 2 from prior treatments, active autoimmune disease requiring systemic treatment, or a history of noninfectious pneumonitis/interstitial lung disease requiring steroids.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Durvalumab 1500 mg administered every 4 weeks (Q4W) concurrently with SOC cCRT, followed by consolidation durvalumab therapy until progressive disease (PD) or specific discontinuation criteria are met. **Route of Administration:** Intravenous (IV) infusion. **Duration of Treatment:** Maximum of up to 54 weeks (or until disease progression or unacceptable toxicity).

5.2 Concomitant & Prohibited Therapy Permitted: Standard of care concurrent chemoradiotherapy (cCRT). Chemotherapy regimens include Cisplatin (Preferred name: cisplatin; Trade name: Platinol; ATC Code: L01XA01) + Etoposide, Carboplatin + Paclitaxel, Pemetrexed + Cisplatin, or Pemetrexed + Carboplatin. **Prohibited:** Concurrent administration of other investigational anti-cancer therapies, live attenuated vaccines, or chronic systemic immunosuppressive corticosteroids.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Tumor Staging, ECOG Assessment
Baseline	Day 0	Randomization (2:1), First IV Dose of Durvalumab/Placebo alongside cCRT
Interim	Q4W	Durvalumab/Placebo IV Infusion, Chemoradiotherapy administration, Safety Labs, Efficacy Assessment (RECIST)
End of Study	Up to Day 1960	Final Vital Signs, Exit Interview, IP Accountability, Long-term Survival Follow-up

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Progression-Free Survival (PFS); time from randomization until objective disease progression or death from any cause. **Measurement Method:** Blinded Independent Central Review (BICR) using RECIST v1.1.

7.2 Secondary & Exploratory Endpoints - Overall Survival (OS) - Objective Response Rate (ORR) and Duration of Response (DoR) - Percentage of Participants Alive at 24 Months From Randomization (OS24) - Incidence of adverse events (AEs) and immune-mediated AEs, particularly radiation pneumonitis

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring of treatment-emergent adverse events, with particular focus on infections, immune-mediated adverse events (imAEs), and pneumonitis/radiation pneumonitis during the concurrent chemoradiation phase. **Serious Adverse Events (SAEs):** Reported within 24 hours. SAEs monitored closely given the overlap of radiation toxicity and immunotherapy toxicity (e.g., Grade 3/4 AEs, fatal events). **Data Monitoring Committee (DMC):** Independent Data Monitoring Committee utilized to review unblinded safety and efficacy data continuously throughout the study.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) for primary efficacy analyses (PFS and OS); Safety Set for all patients who received at least one dose of the study drug. **Sample Size Justification:** Powered to detect a statistically significant improvement in PFS and OS for the Durvalumab + cCRT arm versus Placebo + cCRT arm (N=328). **Handling of Missing Data:** Standard time-to-event censoring rules applied (e.g., right-censoring at the date of last evaluable disease assessment for PFS).

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Routine onsite and remote monitoring visits to ensure protocol adherence, safety reporting, and RECIST v1.1 evaluation compliance. **Audit Trail:** Electronic Case Report Form (eCRF) locking and central imaging repositories for BICR tumor assessments.

11. Study Identifiers & Governance

Primary ID: D933KC00001 **Registry Number:** NCT03519971 **Posting ID:** 1103866201621950379 **Sponsor/Lead Organization:** AstraZeneca **Study Title:** Study of Durvalumab Given With Chemoradiation Therapy in Patients With Unresectable Non-small Cell Lung Cancer **Official Regulatory Title:** A Phase III, Randomized, Double-Blind, Placebo-Controlled, Multicenter Study of Durvalumab Given Concurrently with Platinum-Based Chemoradiation Therapy in Patients with Unresectable, Stage III Non-Small Cell Lung Cancer

12. Clinical Framework (Specific)

Primary Condition: Metastatic Non-Small Cell Lung Cancer (Locally advanced, unresectable Stage III NSCLC) **Disease Area / Preferred UMLS Name:** Non-Small Cell Lung Cancer / Metastatic non-small cell lung cancer **Diagnosis Threshold:** Histologically or cytologically confirmed Stage III NSCLC **Medical Methodology:** Concurrent Chemoradiotherapy (cCRT) combined with Immunotherapy **Optimization Target:** Improvement in Progression-Free Survival (PFS) and Overall Survival (OS)

13. Study Procedures & Methodology

Management Pathway: Standard platinum-based cCRT (e.g., Cisplatin+Etoposide or Carboplatin+Paclitaxel) administered simultaneously with Durvalumab/Placebo, followed by consolidation Durvalumab/Placebo. **Education Protocol (HCP):** Aggressive monitoring and treatment algorithms for overlapping toxicities, specifically immune-mediated and radiation-induced pneumonitis. **Education Protocol (Patient):** Self-monitoring for respiratory symptoms (shortness of breath, cough) and signs of severe infection. **Quality Audit Mechanism:** Blinded Independent Central Review (BICR) for all radiographic imaging to confirm PFS endpoints.

14. Observation & Follow-Up Schedule

Total Duration: Up to 1960 Days (~5.3 years) for survival follow-up. **Onsite Visit Cadence:** Every 4 Weeks (Q4W) for study drug infusion and toxicity assessment. **Remote Monitoring Frequency:** Routine survival follow-up contacts after study drug discontinuation. **Checklist Review Interval:** Regular radiographic imaging (CT/MRI) per RECIST v1.1 protocol timelines (e.g., every 8-12 weeks).

15. Sign-off & Approval

Role	Name	Signature	Date		:---	:---	:---	:---
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]					Lead
Statistician	[Name]	_____	[DD-MMM-YYYY]					